

IDENTIFICATION OF THE MOST COMMON BARRIERS IN DIABETIC FOOT CLINICS IN BELGIUM

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On behalf of the Initiative for Quality improvement and Epidemiology in multidisciplinary Diabetic Foot Clinics (IQED-Foot) Group of Experts

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BACKGROUND

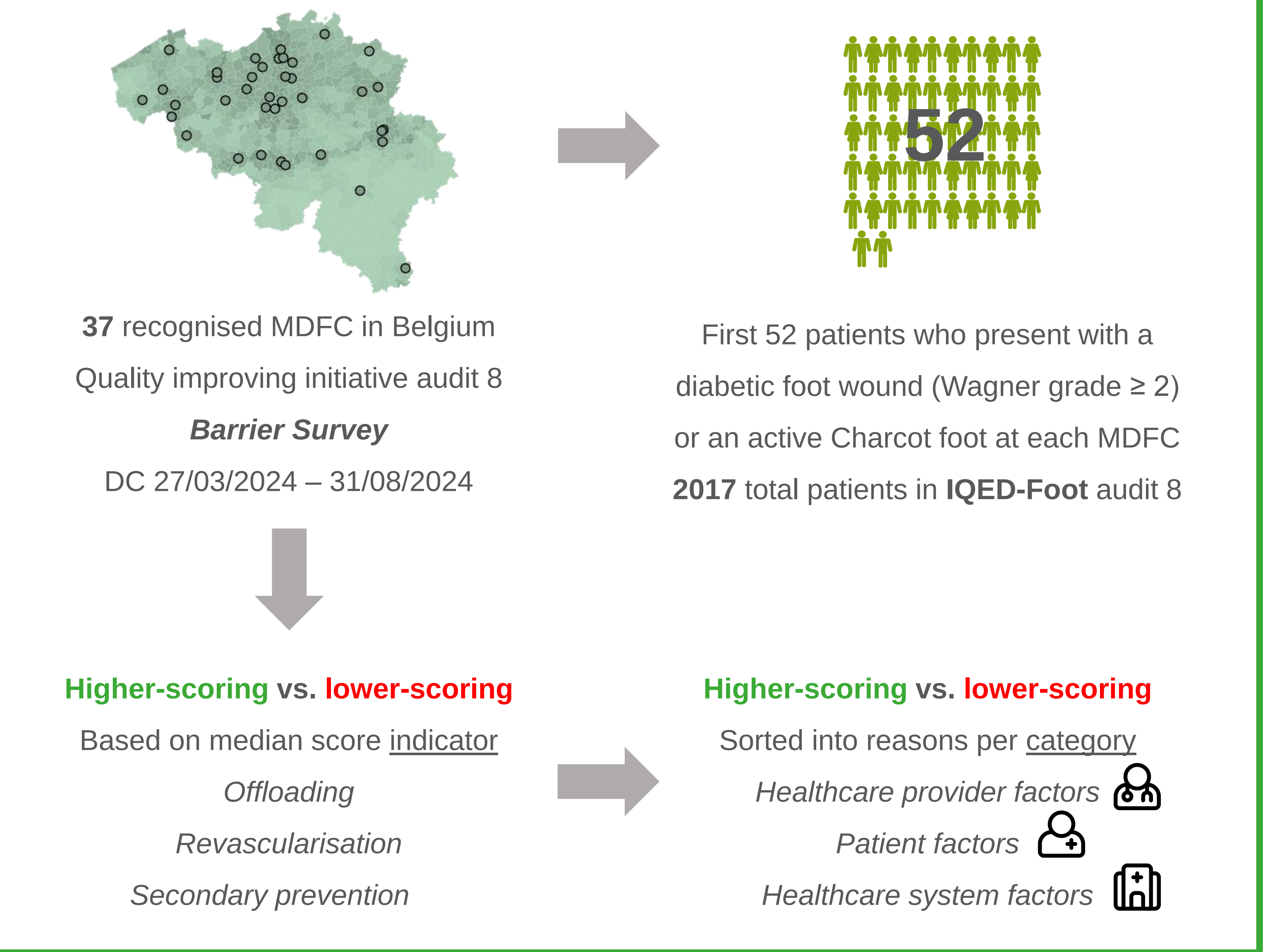
Diabetic foot complications pose a major burden on individuals' quality of life. In Belgium, these problems are addressed by specialised teams within multidisciplinary diabetic foot clinics (MDFC). Since 2005, the national **IQED-Foot** registry has been dedicated to monitoring and enhancing the quality of care provided to diabetic foot patients. Through biennial data collection, **IQED-Foot** generates a national report and offers individualised feedback through benchmarking, supporting healthcare providers in their continuous efforts to improve diabetic foot care outcomes.

IQED-Foot reveals however large variations in diabetic foot care between different MDFC, particularly in terms of:

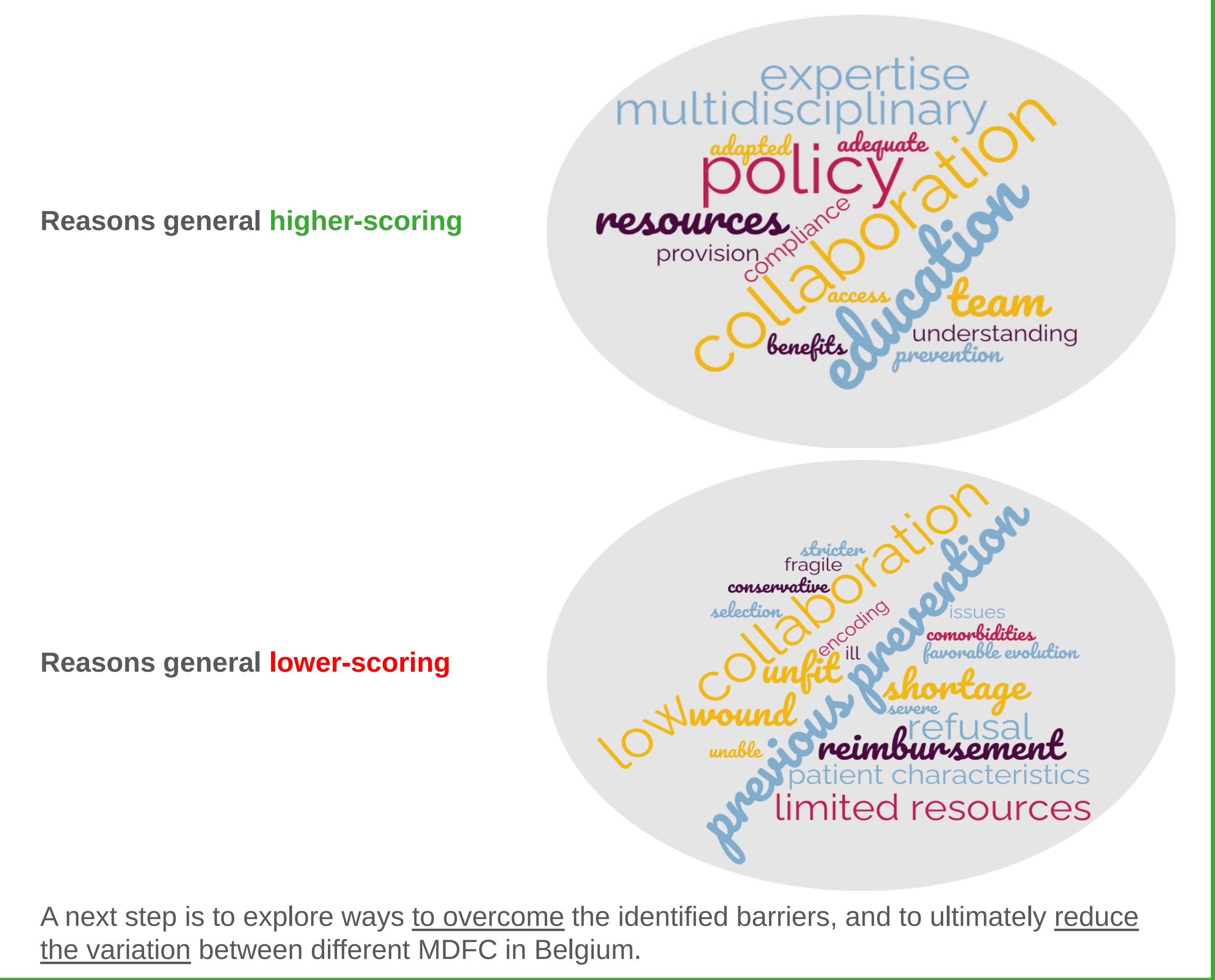
- Offloading
- Revascularisation
- Secondary prevention

To better understand the reasons for these variations, a survey was launched in 2024 aiming to identify the most common barriers to proper diabetic foot care in Belgian MDFC.

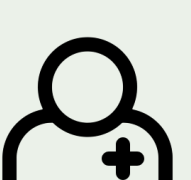
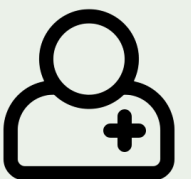
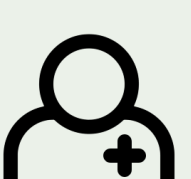
METHODS



CONCLUSION



RESULTS

OFFLOADING		
	higher-scoring	lower-scoring
	• Strong offloading policy • Patient education • Expertise	• Prevention policy • Limited trained personnel • Registration issues
	• Compliance • Awareness	• Refusal • Patient characteristics
	• Intensive multidisciplinary collaboration • Easier access to resources	• Low multidisciplinary collaboration • Reimbursement policy
Examples from practice	Optimal pursuit of offloading Commit to patient awareness Presence orthopaedic experts	No need for complex offloading Shortage of nursing staff Incomplete registrations due to change of team Bedridden, palliative patients Orthopaedic experts not present Expensive orthopaedic shoes
REVASCULARISATION		
	higher-scoring	lower-scoring
	• Strong revascularisation policy • Expertise	• Conservative approach • Limited trained personnel
	• Treating patients with critical limb ischemia	• Patient characteristics • Favourable wound evolution without revascularisation
	• Intensive multidisciplinary collaboration • Adequate resources	• Low multidisciplinary collaboration
Examples from practice	Advice and experience from vascular surgeons Explain importance to patients Close cooperation vascular surgery	Shortage of vascular surgeons or radiologists Non-surgical policy adopted first Fragile, severely ill patients Separate management of vascular patients (other department)
SECONDARY PREVENTION		
	higher-scoring	lower-scoring
	• Strong secondary prevention policy • Patient education	• Previous secondary prevention policy • Limited trained personnel
		• Refusal • Patient characteristics
	• Intensive multidisciplinary collaboration	• Low multidisciplinary collaboration • Reimbursement policy • Prescription policy
Examples from practice	Provision of orthopaedic insoles/shoes Orthopaedic shoes always advised Presence orthopaedic experts	Patients already have orthopaedic shoes Bedridden, palliative patients Two-year renewal, no refund for indoor footwear Diabetologists cannot prescribe orthopaedic insoles/shoes